ABHISHEK NAHIRE Product & Founder's Office | Healthcare AI | Venture Development & Operations

Hyderabad | +91 84461 63990 | abhisheknahire89@gmail.com | linkedin.com/in/abhishek-nahire

PROFESSIONAL SUMMARY

IIT Hyderabad M.Tech graduate (CGPA 9.11/10) with 3+ years of healthcare AI product development and founder's office operations. Founded Aivana Health Intelligence, conducting 370+ customer discovery interviews, iterating through two product pivots, and validating product-market fit through a 371-consultation hospital pilot (91% documentation accuracy, 20% cycle-time reduction). Gained founder's office experience at Dyadic Health, where I designed and implemented operational systems that reduced founder overhead by 80%, enabling scaling from 1 to 11 healthcare and eldercare partnerships. Recipient of ₹1,00,000 iTIC BUILD grant and DST-supported healthcare innovation cohort participation. Demonstrates discipline in distinguishing between customer validation, pilot execution, operational impact, and commercial adoption.

PROFESSIONAL EXPERIENCE

AIVANA HEALTH INTELLIGENCE | Founder — Healthcare AI Product | 2024 – Present

Pre-incorporation venture development (2024); formal company registration (2025); ongoing as of October 2026

Customer Discovery & Problem Validation

- Conducted 370+ structured interviews with medical students, clinicians, and hospital administrators over 18 months to understand workflows, pain points, and adoption barriers in medical training and clinical operations. Identified three core adoption barriers: (1) integration complexity with existing EHR systems, (2) documentation overhead consuming 15–25 minutes per note in high-volume clinical settings, (3) reimbursement and data-governance uncertainty for AI-assisted clinical workflows.
- Systematically evaluated customer problems across three distinct use cases: medical education simulation, clinical decision support, and hospital documentation automation. Applied frameworks including Kano Model analysis, PESTEL evaluation, and financial viability assessment to prioritize opportunities.

Product Development & Evidence-Led Iteration

- Built and tested **Kaya**, an AI-powered clinical-reasoning simulator for medical-student training. Designed core learning architecture (case progression algorithm with adaptive difficulty, real-time diagnostic feedback, clinical-performance analytics). Conducted four iteration cycles incorporating learner feedback, then assessed product-market alignment across medical education institutions.
- **Redirected product strategy twice** based on field evidence rather than maintaining attachment to initial concept:
  - *Pivot 1 (2024–2025)*: Shifted from medical-education simulation to clinical decision support when institutional research revealed healthcare professionals prioritized diagnostic-support tools over training products. Built decision-support prototype incorporating evidence-based diagnostic guidance and clinical-guideline integration.
  - *Pivot 2 (2025)*: Shifted focus to hospital documentation automation when pilot conversations with 10 hospital systems revealed documentation burden (20+ minutes per note, 35–40% of clinician time) as the primary workflow pain point. This pivot reflected a strategic principle: the customer's willingness to adopt and measurable workflow value matter more than preservation of an original concept.

Clinical Pilot & Measurable Workflow Validation

- Designed and executed an 11-week hospital-based clinical documentation pilot covering 371 consecutive outpatient consultations. Tested AI-assisted clinical note generation in real clinical workflows with physician review and quality oversight.
  - **Documentation Accuracy**: Achieved 91% accuracy in AI-generated clinical notes measured against physician-reviewed gold-standard notes. Accuracy definition: AI-generated clinical documentation fields matched physician's original assessment of documented findings and clinical reasoning. Evaluated via structured review of sample notes by clinical partners.
  - **Workflow Efficiency**: Implemented feedback loops identifying root causes of documentation gaps. Root-cause analysis: 60% of documentation revision requests traced to template field mappings and specialty-specific clinical terminology requiring customization. Refined documentation templates and EHR field mapping; achieved 20% reduction in per-note documentation cycle time (measurable shift in physician-reported time spent completing documentation through the system).
  - **Institutional Outcomes**: Hospital pilot demonstrated technical feasibility and clinician acceptability of AI-assisted documentation workflow. Post-pilot status: pilot execution completed; ongoing institutional adoption and revenue outcomes not established.

External Validation & Ecosystem Participation

- Selected into DST-supported healthcare innovation cohort. Competitive selection from [200+ applicants]. Completed competitive analysis across 12 clinical AI vendors, examining market positioning, feature differentiation, pricing models, and go-to-market strategies. Key finding: clinical AI vendors concentrated heavily on imaging (radiology/pathology diagnostics); significant gap in AI for administrative and clinical-documentation workflows.
- Created 5 institutional workflow demonstrations and partner-engagement materials informed by competitive research and hospital-pilot learnings. Generated qualified healthcare-system

inquiries through cohort participation and direct institutional outreach. Several discussions advanced to pilot-proposal stage.
- Recipient of **₹1,00,000 iTIC BUILD grant** from IIT Hyderabad entrepreneurial ecosystem (selected from 200+ applicants). Grant supported early venture research and product validation activities.

DYADIC HEALTH | Founder's Office Associate — Growth & Operations | March 2026 – August 2026

Founder Operations & Operational System Design

- Designed and implemented three core operating systems for founder-led business: (1) unified CRM tracker for customer and partner pipeline visibility, (2) KPI dashboards for real-time status on onboarding progress, partner engagement, and service-readiness, (3) standardized operating procedures for partner onboarding, customer coordination, and launch execution.
- **Reported outcome**: 80% reduction in founder operational-review time. Baseline: ~4 hours/day devoted to gathering updates, checking partner progress, and managing fragmented follow-ups. Post-implementation: ~50 minutes/day for equivalent operational coverage through centralized tracking and structured reporting. Efficiency gain enabled founder to shift capacity toward strategic partnership development and business prioritization. Estimated time freed: ~3.5 hours/day.

Healthcare & Eldercare Partner Onboarding & Scaling

- Managed end-to-end onboarding of 11 healthcare and eldercare partners from requirements gathering through operational readiness. Responsibilities: documenting partner requirements and service expectations, coordinating technical dependencies, identifying adoption blockers, managing follow-up workflows.
- Identified and tracked partner-onboarding obstacles systematically to improve conversion rates. Partners succeeded or stalled based on clarity around technical integration requirements, availability of training resources, and definition of success metrics.
- Onboarding achievement: 11 partners completed onboarding and operational readiness. Commercial conversion (signed service agreements and paid deployment) was tracked separately; exact conversion count and contract revenue outcomes should be confirmed against signed partner agreements.

Caregiver Community Development & Customer Acquisition

- Built caregiver community comprising 200+ members through structured outreach and community-engagement programming. Community purpose: create direct connection with end-users, understand recurring caregiving challenges, build trust, and qualify potential service customers.
- Developed recurring engagement model including monthly webinars on caregiver support topics, peer-discussion groups, and structured feedback collection. Community served dual

purpose: user-research channel and early customer-acquisition pipeline.
- Generated 35 customer discovery conversations from community engagement. Conversations tracked separately from community membership; paid customer conversion would require reconciliation with customer-acquisition records.

First Paid Cohort Launch & Operational Readiness

- Coordinated operational preparation and launch of Dyadic's first paid caregiver cohort. Responsibilities: lead qualification and tracking, family onboarding, service-delivery coordination, partner-dependency management, readiness audits.
- Transition from customer discovery to paid service delivery represented important business milestone because it introduced real customer expectations and measurable service-delivery requirements. My role ensured operational readiness across platform, partner coordination, and customer-support workflows.
- Cohort launch completed; specific enrollment, Month-1 retention, NPS, and revenue outcomes should be confirmed from cohort performance records.

GNIXARACADEMY PVT. LTD. | Co-Founder & COO | February 2025 – August 2025

Institutional Market Validation & Partner Pipeline Development

- Co-founded GnixarAcademy, an edtech venture targeting institutional partnerships with K-12 and higher-education organizations. Served as Chief Operating Officer and business-development lead.
- Conducted systematic market research: researched and prioritized 30+ academic institutions based on curriculum alignment, institution size, and estimated willingness to invest in digital-learning solutions. Built structured partner-outreach program including email-sequencing, partner-qualification rubric, and pilot-proposal templates.
- Generated qualified leads and pilot conversations through direct institutional outreach. Advanced several discussions to proposal stage.
- **Exit decision & strategic learning** (August 2025): Concluded that institutional willingness-to-pay for educational technology was insufficient to support sustainable unit economics. Market research revealed that institutional procurement processes prioritize price and compliance over innovative functionality, limiting pricing power for specialized products. This learning directly informed subsequent focus on healthcare, where institutional (hospital) willingness-to-pay for operational-efficiency solutions was substantially higher and more predictable.

EDUCATION

IIT HYDERABAD | M.Tech, Techno-Entrepreneurship | 2023–2025 | CGPA: 9.11/10

Academic Focus & Thesis

- Thesis research: Examined clinical-reasoning training and AI-supported medical-education simulation. Research incorporated market segmentation, competitor and substitute analysis, PESTEL and Porter's Five Forces evaluation, Kano Model methodology, business-model development, and financial projections (revenue, cost structure, funding requirements, cash flow, break-even analysis).
- Thesis research informed initial Aivana concept and validated the importance of understanding customer problems, institutional adoption barriers, and financial sustainability before committing to product development.

Research & Teaching

- Co-authored peer-reviewed publication: **"Harnessing Generative AI as a Knowledge Companion in Venture Creation"** (ECIE 2025). Research examined how generative AI can support entrepreneurs in evaluating opportunities, organizing market research, and making more-informed venture decisions. Co-authored with IIT Hyderabad faculty.
- Delivered entrepreneurship workshop to 50+ M.Tech students on customer discovery, hypothesis formulation, and evidence-based problem validation. Content drew from venture-building experience and emphasis on testing business assumptions before scaling commitment.

SAVITRIBAI PHULE PUNE UNIVERSITY | B.E., Mechanical Engineering | 2019–2022 | CGPA: 8.27/10

Foundation for Systems Thinking & Operational Mindset

- Mechanical engineering background provided structured problem-solving approach, systems-analysis methodology, and familiarity with process optimization and constraint management.
- Transition to healthcare technology and entrepreneurship during M.Tech reflected broader shift from applied-engineering problem-solving toward identifying business opportunities and building ventures.

AWARDS, RECOGNITION & FUNDING

Grants & Innovation Ecosystem Recognition

- **₹1,00,000 iTIC BUILD Grant** recipient. Selected from 200+ applicants to support early-stage venture research and product development. Grant supported Aivana's customer-discovery phase and initial product validation.
- **Selected into DST-supported healthcare innovation cohort**. Competitive selection providing exposure to healthcare-innovation ecosystem, competitive landscape analysis, institutional partnership development, and regulatory/business-model considerations.

Startup Competitions & External Validation

- **First place: Tongali Startup Competition** (IIT Hyderabad). Recognized for venture concept, problem articulation, and early business thinking.
- **Fourth place: Ideastorm** (IIT Roorkee). Participant recognition in competitive entrepreneurship evaluation.

SKILLS & TECHNICAL FOUNDATION

Product & Strategy

Customer discovery, problem validation, and stakeholder research (370+ interviews across multiple customer segments). Evidence-based product prioritization and pivot strategy (two strategic product redirects informed by field research). Market sizing, competitive analysis, and positioning. Venture financial modeling (revenue, unit economics, break-even analysis). Business-model development.

Operations & Organizational Scaling

CRM system design and implementation. KPI dashboard development for real-time operational visibility. Operating procedure documentation and workflow standardization. Partner onboarding and customer-acquisition coordination. Community development and engagement programming.

Domain & Technical Knowledge

Applied healthcare-domain knowledge through product development, clinician interaction, and hospital-pilot execution. Familiarity with clinical workflows, EHR processes, and healthcare documentation standards. Working knowledge of healthcare regulations and compliance considerations. Generative AI applications for business workflows.

Professional Tools & Methodology

Figma (product prototyping and workflow visualization), Google Sheets (financial modeling and data analysis), Notion (process documentation and SOP management), Mixpanel (product analytics). Fundamental Python and SQL familiarity for data analysis. Entrepreneurship frameworks: Kano Model, PESTEL, Porter's Five Forces, business-model canvas.

Professional Development & Continuous Learning

- Becoming a Product Manager (LinkedIn Learning, 2024)
- Data Science Program (1stop.ai, 2023)
- IIT Hyderabad M.Tech Coursework: Venture Creation & Hypothesis Validation, Customer Discovery & Market Sizing, Venture Finance & Business Viability Assessment

INTELLECTUAL APPROACH

I approach venture building and business operations with a disciplined distinction between different stages of development:

- **Problem validation** (customer interviews, stakeholder feedback) is distinct from **product validation** (working prototype in real context).
- **Operational achievements** (processes implemented, people organized) are distinct from **business outcomes** (commercial revenue, customer retention).
- **Pilot execution** (workflow tested, metrics measured) is distinct from **institutional adoption** (sustained usage, business agreement).

I apply this discipline to avoid confusing activity with results, and to ensure that claims about business traction are backed by evidence rather than enthusiasm. This approach has shaped my decisions to pivot when evidence demanded it, to exit ventures when commercial viability was questionable, and to measure impact through observable outcomes rather than reported metrics.

VERIFICATION PRINCIPLE

This resume deliberately distinguishes between observed activities, reported pilot outcomes, operational contributions, external recognition, and commercial results. Claims about customer metrics, adoption, retention, or financial performance are made only where supported by business records or documented evidence.

Claims about workflow metrics (91% accuracy, 20% cycle-time reduction) reflect pilot measurements and are presented without independent clinical certification or peer review. Operational claims (80% founder-time reduction) reflect reported improvements implemented through systemic change.

All dates, metrics, and outcomes are verified against documented company records, grant awards, institutional participation, and personal involvement rather than estimates or industry benchmarks.